

AUSTEDO® (deutetrabenazine) Tablets PI Explorer

Introduction

The AUSTEDO® (deutetrabenazine) Tablets PI Explorer is intended to increase your understanding of AUSTEDO by presenting in-depth information within the full AUSTEDO prescribing information (PI). This training tool is for internal use only.

A variety of call-out icons are located throughout this guide. Each icon explains additional information.

 **Clinical Concept:** provides background clinical information explaining why a PI section may be important to a healthcare provider (HCP)

 **Key Point:** explains how information in a PI section supports a key message and how to use the content in selling situations

 **Safety Note:** explains key safety information

 **FAQ:** describes how the information in a PI section can be used to respond to a frequently asked question (FAQ)

 **Field Note:** describes how to apply information from the PI in the field

Glossary terms can be found at the end of this document. Please review their definitions.

This guide is not intended to represent the full extent of the information you should know from the PI. It is important that you read through the PI completely.

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PI Page 1



HIGHLIGHTS OF PRESCRIBING INFORMATION

These highlights do not include all the information needed to use AUSTEDO safely and effectively. See full prescribing information for AUSTEDO.

AUSTEDO® (deutetrabenazine) tablets, for oral use
Initial U.S. Approval: 2017

WARNING: DEPRESSION AND SUICIDALITY IN PATIENTS WITH HUNTINGTON'S DISEASE

See full prescribing information for complete boxed warning.

- Increases the risk of depression and suicidal thoughts and behavior (suicidality) in patients with Huntington's disease (5.1)
- Balance risks of depression and suicidality with the clinical need for treatment of chorea when considering the use of AUSTEDO (5.1)
- Monitor patients for the emergence or worsening of depression, suicidality, or unusual changes in behavior (5.1)
- Inform patients, caregivers, and families of the risk of depression and suicidality and instruct to report behaviors of concern promptly to the treating physician (5.1)
- Exercise caution when treating patients with a history of depression or prior suicide attempts or ideation (5.1)
- AUSTEDO is contraindicated in patients who are suicidal, and in patients with untreated or inadequately treated depression (4, 5.1)

RECENT MAJOR CHANGES

Dosage and Administration (2.1) (removal) 12/2020
Warnings and Precautions (5.3) 12/2020

INDICATIONS AND USAGE

AUSTEDO is a vesicular monoamine transporter 2 (VMAT2) inhibitor indicated for the treatment of:

- Chorea associated with Huntington's disease (1)
- Tardive dyskinesia in adults (1)

DOSAGE AND ADMINISTRATION

	Initial Dose	Recommended Dose	Maximum Dose
Chorea associated with Huntington's disease	6 mg/day	6 mg– 48 mg/day	48 mg/day
Tardive dyskinesia	12 mg/day	12 mg– 48 mg/day	48 mg/day

- Titrate at weekly intervals by 6 mg per day based on reduction of chorea or tardive dyskinesia, and tolerability, up to a maximum recommended daily dosage of 48 mg (24 mg twice daily) (2.1)
- Administer total daily dosages of 12 mg or above in two divided doses (2.1)
- Administer with food (2.1)
- Swallow tablets whole; do not chew, crush, or break (2.1)

- If switching patients from tetrabenazine, discontinue tetrabenazine and initiate AUSTEDO the following day. See full prescribing information for recommended conversion table (2.2)
- Maximum recommended dosage of AUSTEDO in poor CYP2D6 metabolizers is 36 mg per day (i.e., 18 mg twice daily) (2.4, 8.7)

DOSAGE FORMS AND STRENGTHS

Tablets: 6 mg, 9 mg, and 12 mg (3)

CONTRAINDICATIONS

- Suicidal, or untreated/inadequately treated depression in patients with Huntington's disease (4, 5.1)
- Hepatic impairment (4, 8.6, 12.3)
- Taking reserpine, MAOIs, tetrabenazine (XENAZINE®), or valbenazine (4, 7.2, 7.3, 7.6)

WARNINGS AND PRECAUTIONS

- QT Prolongation: Avoid use in patients with congenital long QT syndrome or with arrhythmias associated with a prolonged QT interval (5.3)
- Neuroleptic Malignant Syndrome (NMS): Discontinue if this occurs (5.4)
- Akathisia, agitation, restlessness, and parkinsonism: Reduce dose or discontinue if this occurs (5.5, 5.6)
- Sedation/somnolence: May impair the patient's ability to drive or operate complex machinery (5.7)

ADVERSE REACTIONS

Most common adverse reactions (>8% of AUSTEDO-treated patients with Huntington's disease and greater than placebo): somnolence, diarrhea, dry mouth, and fatigue (6.1)

Most common adverse reactions (that occurred in 4% of AUSTEDO-treated patients with tardive dyskinesia and greater than placebo): nasopharyngitis and insomnia (6.1)

To report SUSPECTED ADVERSE REACTIONS, contact Teva Pharmaceuticals at 1-888-483-8279 or FDA at 1-800-FDA-1088 or www.fda.gov/medwatch.

DRUG INTERACTIONS

- Concomitant use of strong CYP2D6 inhibitors: Maximum recommended dose of AUSTEDO is 36 mg per day (18 mg twice daily) (2.3, 7.1)
- Alcohol or other sedating drugs: May have additive sedation and somnolence (7.5)

USE IN SPECIFIC POPULATIONS

Pregnancy: Based on animal data, may cause fetal harm (8.1)

See 17 for PATIENT COUNSELING INFORMATION and Medication Guide.

Revised: 12/2020

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Key Point

Section 1: Highlights of Prescribing Information

The PI begins with the “Highlights of Prescribing Information” section. This section is designed to summarize the key information from the full PI and therefore shows the most important information to point out to your HCPs.



Field Note

Section 1: Highlights of Prescribing Information

While “Highlights of Prescribing Information” points out the most crucial pieces of information, it is important to note that these highlights do not contain ALL the information needed to safely and effectively prescribe and use AUSTEDO, and the HCP should turn to the full PI to get all the relevant information. Therefore, it is also important that you fully understand the content of the full PI.



Safety Note

Section 1: Highlights of Prescribing Information

Certain contraindications or serious warnings that may lead to death or serious injury, as determined primarily in clinical data, may be required by the FDA to be described in a Boxed Warning summarized at the beginning of the Highlights section with the complete text at the beginning of the Full Prescribing Information. The AUSTEDO PI carries a boxed warning regarding an increased risk of depression and suicidality in patients with Huntington’s disease (HD).

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No annotations or callouts

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FULL PRESCRIBING INFORMATION

WARNING: DEPRESSION AND SUICIDALITY IN PATIENTS WITH HUNTINGTON'S DISEASE

AUSTEDO can increase the risk of depression and suicidal thoughts and behavior (suicidality) in patients with Huntington's disease. Anyone considering the use of AUSTEDO must balance the risks of depression and suicidality with the clinical need for treatment of chorea. Closely monitor patients for the emergence or worsening of depression, suicidality, or unusual changes in behavior. Patients, their caregivers, and families should be informed of the risk of depression and suicidality and should be instructed to report behaviors of concern promptly to the treating physician.

Particular caution should be exercised in treating patients with a history of depression or prior suicide attempts or ideation, which are increased in frequency in Huntington's disease. AUSTEDO is contraindicated in patients who are suicidal, and in patients with untreated or inadequately treated depression [see Contraindications (4) and Warnings and Precautions (5.1)].

1 INDICATIONS AND USAGE

AUSTEDO® is indicated for the treatment of:

- chorea associated with Huntington's disease [see Clinical Studies (14.1)]
- tardive dyskinesia in adults [see Clinical Studies (14.2)]

2 DOSAGE AND ADMINISTRATION

2.1 Dosing Information

The dose of AUSTEDO is determined individually for each patient based on reduction of chorea or tardive dyskinesia and tolerability. When first prescribed to patients who are not being switched from tetrabenazine (a related VMAT2 inhibitor), the recommended starting dose of AUSTEDO is 6 mg administered orally once daily for patients with Huntington's disease and 12 mg per day (6 mg twice daily) for patients with tardive dyskinesia.

- The dose of AUSTEDO may be increased at weekly intervals in increments of 6 mg per day to a maximum recommended daily dosage of 48 mg.
- Administer total daily dosages of 12 mg or above in two divided doses.
- Administer AUSTEDO with food [see Clinical Pharmacology (12.3)].
- Swallow AUSTEDO whole. Do not chew, crush, or break tablets.

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Safety Note

Section 2: Full Prescribing Information

As already shown in the Highlights of Prescribing Information, the AUSTEDO PI carries a boxed warning regarding an increased risk of depression and suicidality in patients with HD. The PI for the related compound, tetrabenazine, carries the same boxed warning.

The boxed warning states that AUSTEDO can increase the risk of depression and suicidal thoughts and behavior in patients with HD, and that this increased risk must be balanced against the clinical need for treatment of chorea. This is particularly important because depression is a common symptom associated with HD, occurring in about 40% of patients. In addition, the rates of suicide are higher among HD patients than in the general population.

Patients and their caregivers should be informed about this increased risk and the patient closely monitored for emergence or worsening of depression or suicidality, particularly if the patient has a history of depression or suicide attempts.

The boxed warning also states that AUSTEDO is contraindicated in patients who are suicidal, and in patients with untreated or inadequately treated depression.



Key Point

Section 2: Full Prescribing Information

You should know that AUSTEDO is indicated in adults for the treatment of chorea associated with HD and for the treatment of tardive dyskinesia (TD).

In your discussions with HCPs, you can point out that AUSTEDO is only the second medication approved by the FDA for chorea associated with HD, with its parent compound, tetrabenazine, being the only previously approved drug. Although many other medications are also being used for treatment of chorea in patients with HD (eg, antipsychotics, glutamate or NMDA receptor antagonists, benzodiazepines, cannabinoids), these agents are used off-label for such patients.

You may also point out that AUSTEDO is the first and only medication approved to treat both TD in adults and chorea associated with HD.



FAQ

Section 2: Dosage and Administration

If you're asked about appropriate dosing for patients with HD or TD, this section provides some key important points, but Section 2 should always be consulted for more complete dosing information. Note that the dosing regimen depends on the patient's previous treatment history. Different dosing recommendations exist for patients who are not switching to AUSTEDO from tetrabenazine and those who are switching. **Section 2.1** details dosing information for patients who are not being switched from tetrabenazine; **Section 2.2** provides dosing information for patients who are being switched from tetrabenazine.

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Clinical Concept

Section 2.1: Dosing Information

It is important to point out to your HCPs that the dose of AUSTEDO must be individually adjusted for each patient. When a patient is first prescribed AUSTEDO, the dose should slowly be increased over several weeks to identify a dose that reduces chorea or TD but does not induce adverse reactions. This is particularly important because the severity of movement disorders differs greatly among patients and can change over time, necessitating repeated assessments and, if necessary, dose adjustments. For example, chorea typically decreases during the course of the disease and is replaced by akinesia (absence or lack of control of voluntary movements), rigidity, and spasticity (increased muscle resistance to passive stretching), which may not respond to treatment for chorea.



Clinical Concept

Section 2.1: Dosing Information

For patients who are not being switched from tetrabenazine, the recommended starting dose is 6 mg once daily for patients with HD or 6 mg twice daily (for a total daily dose of 12 mg) for patients with TD.



Clinical Concept

Section 2.1: Dosing Information

If adequate chorea or TD control is not achieved with the starting dosage, the dosage may be increased at weekly intervals in increments of 6 mg/day, until a dose is identified that reduces chorea or TD but is well tolerated. This process is similar to that for tetrabenazine, which should also be titrated slowly. AUSTEDO dosages of 12 mg/day and greater must be administered in 2 divided doses.

The maximum recommended dose for AUSTEDO is 48 mg/day, administered as 2 divided doses of 24 mg each. In patients who are poor CYP2D6 metabolizers or are taking strong CYP2D6 inhibitors, the total daily dose of AUSTEDO should not exceed 36 mg (18 mg in 2 divided doses).



Field Note

Section 2.1: Dosing Information

Note that, even at higher doses, AUSTEDO is administered in a regimen of 2 daily doses.



Clinical Concept

Section 2.1: Dosing Information

AUSTEDO should be taken with food, and patients should be advised that the tablets should be swallowed whole and should not be chewed, crushed, or broken. This is particularly important because many patients have swallowing problems (dysphagia) and, therefore, may be tempted to crush or break tablets to facilitate swallowing.